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**Centers for Medicare & Medicaid Services, Department of Health and Human Services, Attention: CMS-0042-NC, P.O. Box 8013, Baltimore, MD 21244-8013.**

**RE: Request for Information; Health Technology Ecosystem**

Verily welcomes the opportunity to provide comments on the Health Technology Ecosystem Request for Information. Digital health technologies have a fundamental role to play in modernizing the health data ecosystem and facilitating a transition from reactively managing disease to proactively preventing illness and chronic conditions. The priorities established by the Administration, including the Executive Orders on Artificial Intelligence (AI) leadership,[1] the establishment of the Make America Healthy Again Commission,[2] and the recent strategic framework issued by the Centers for Medicare & Medicaid Services (CMS) Innovation Center[3] demonstrate the Administration's commitment to fostering responsible technological advancement and innovation in healthcare delivery. We believe forthcoming policy can and should build on the progress made by stakeholders implementing the 21st Century Cures Act (P.L. 114-255) to enhance access to and exchange of data, accelerate precision and preventative care, and improve health outcomes.

Verily is an Alphabet data platform and AI company that is purpose-built for precision health. We combine deep clinical, regulatory, and technology expertise to provide our customers with patient-centric solutions that make healthcare more personal and precise. Verily solutions are designed to make healthcare data AI-ready and fit for purpose across care, research, and public health. Our extensive experience building innovative technology platforms for data ingestion, curation, and analysis, alongside our commitment to generating high-quality, regulatory-grade real-world data (RWD), positions us to offer valuable perspectives on advancing the collection, standardization, and regulatory acceptance of evidence derived from real-world sources.

In our response, we highlight three key policy objectives that are essential for advancing the digital health ecosystem. These include:

1) **Building a connected technology ecosystem that empowers patients and enables precision care, including preventative care, through multimodal data exchange**
2) **Incentivizing use of digital technology for precise outcomes that enable personalized health and value-based care**
3) **Partnering on an AI regulatory framework for real-world monitoring & payment models**

[1] https://www.whitehouse.gov/presidential-actions/2025/01/removing-barriers-to-american-leadership-in-artificial-intelligence/
[2] https://www.whitehouse.gov/presidential-actions/2025/02/establishing-the-presidents-make-america-healthy-again-commission/
[3] https://www.cms.gov/priorities/innovation/about/strategic-direction

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We believe these areas align with recent Administration initiatives at CMS and the Assistant Secretary for Health Technology/Office of the National Coordinator for Health Information Technology (ASTP/ONC) and will be important enablers for advancing the health technology ecosystem. For clarity and brevity, we have structured our comments through a discussion of these specific policy areas, which broadly align with questions within the RFI.

**Building a Connected Health Technology Ecosystem that Empowers Patients and Enables Precision Care, including Preventative Care, through Multimodal Data Exchange**

While the healthcare sector generates more than 30% of global data annually, only 2% is retained for the following year and only 3% of data generated at hospitals is used to make evidence-based care decisions.[4] Access to electronic health records (EHRs), including through established Health Information Exchanges (HIEs), continues to be a pain point. Additionally, health data generated outside of the traditional care delivery system, such as data from wearables, patient-reported outcomes, symptom logs, genomic studies, and clinical trials is rarely integrated with EHR data. The aggregation and linkage of these multimodal data sources can accelerate progress toward the type of integrated care necessary for a modernized and patient-centered healthcare system.

Advancing the digital ecosystem necessitates an advanced data platform capable of collecting, aggregating, curating, analyzing, and exchanging health data derived from routine care, clinical research, and broader health-related experiences. The Fast Healthcare Interoperability Resources (FHIR) standard will be critical to achieve this aim, as it provides the semantic and technical foundation needed for diverse data exchange. It is imperative to continue incentivizing the widespread adoption of the FHIR standard, and we urge the Administration to continue efforts aimed at incorporating FHIR into its strategic planning. The Draft Federal FHIR Action Plan and Trusted Exchange Framework and Common Agreement (TEFCA) initiative overseen by ASTP/ONC, and Food and Drug Administration's (FDA) recent Notice for public feedback on the use of FHIR in regulatory submissions for drug and biologic products[5] are encouraging policy developments.

We also recommend that CMS and ASTP/ONC continue to support full-spectrum FHIR capability in certified EHRs, as these interfaces will be foundational to the health technology ecosystem. FHIR has tremendous potential to standardize data measurement and quality reporting, which can empower patients to make informed decisions about where they seek care. The Bulk FHIR API, in particular, is invaluable for population health and quality reporting use cases, enabling health systems to efficiently extract cohorts of data for registries or in-depth

[4] https://www.deloitte.com/be/en/Industries/health-care/research/health-data.html
[5]https://www.federalregister.gov/documents/2025/04/23/2025-06967/exploration-of-health-level-seven-fast-healthcare-interoperability-resources-for-use-in-study-data

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analytics to improve care. However, many providers have not yet enabled this critical functionality. To streamline data exchange processes, a centralized, publicly accessible directory, ideally one that lists participating healthcare organizations and their requirements for data exchange, could be developed by building upon existing work to establish an endpoint FHIR Directory.[6]

The continued advancement of HIEs is also needed with a deliberate focus on improving the necessary governance for exchanging health data. Key progress has been made with HIEs, evidenced by ONC's initial effort through an Advanced Notice of Rulemaking to develop a nationwide governance mechanism for trusted exchange in 2012, the longstanding roles of Carequality and CommonWell, and provisions in the 21st Century Cures Act that subsequently established TEFCA.[7,8]

To further advance the goals of TEFCA, CMS and ASTP/ONC should collaborate more closely to incentivize the accessibility of high quality data as part of a seamless data exchange framework. The advantages TEFCA offers are its structure that provides on-ramps to query and retrieve records from any other participant in the network, and the required response mandate for certain exchange use cases. However, improvements are needed to continue scaling the use of the TEFCA data infrastructure.

Lessons learned from the groundbreaking Meaningful Use program could inform opportunities to ensure interoperability by building transparency into governance requirements of information systems. CMS and ASTP could provide guidance to stakeholders implementing TEFCA regarding how requests should be sent as well as how adjudicators of such requests should respond. Further standardizing this communication, including guidance around how to provide sufficient justification and clear explanations of the intended use(s) of the data, and conversely, the interpretation of such requests, will drive predictability and consistency in the network and foster appropriate information sharing.

Enabling patients to access their health data in a single place is one of the most important steps toward empowering individuals to take an active role in their health. While technically feasible in some instances, it remains challenging for patients to access their health data. For example, provider portals may not support data types that should be available via current APIs. There are also usability barriers, including multiple logins and confusing interfaces. This is an area that requires practical solutions, and we support CMS' planned efforts to implement a modern

[6] https://sequoiaproject.org/SequoiaProjectHealthcareDirectoryImplementationGuide/output/Endpoint-fhir-endpoint.html
[7] https://www.govinfo.gov/content/pkg/FR-2012-05-15/pdf/2012-11775.pdf
[8] https://rce.sequoiaproject.org/common-agreement/

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identity verification system for Medicare and other needed solutions to reduce friction with data exchanges.[9]

Even if a patient is able to access their aggregated health data, it may be challenging to obtain useful insights from large volumes of longitudinal health data. We believe it is important for patients to have support interpreting their data and empowering them to achieve their health goals. Ultimately, health data infrastructure is the linchpin to achieve efficient data exchange, aggregation, and informed use of patient data. The governance of TEFCA, including more consistent use of the FHIR standard, could play an important and leading role to this end, and we encourage CMS and ASTP/ONC to continue TEFCA's development.

**Incentivizing Use of Digital Technology for Precise Outcomes that Enable Personalized Health and Value-based Care**

CMS and ASTP/ONC should support novel ways to use multimodal, real-world health data in advancing value-based payment structures that incentivize personalized, preventative health.

The clinical and economic benefits of preventative health are clear, and research suggests that individuals are more likely to engage in preventative health measures when recommendations are *personalized* rather than generic.[10,11] However, enabling truly personalized health at scale requires the use of multimodal, longitudinal health data that provides a holistic view of an individual's health. Verily believes that capturing and effectively leveraging high quality health data to ascertain better outcomes is a prerequisite for scalable preventative health efforts.

Real-world data collected outside of traditional health encounters like home settings can be combined with EHR data to ascertain more precise health outcomes. This type of RWD may include data elements collected from remote monitoring devices, virtual and asynchronous interactions with clinicians that occur between traditional visits, and directly from patients through surveys and symptom logs. Data elements that give a more comprehensive picture of patient health can then be translated into specific preventative health recommendations for patients *and* into clinical outcomes as part of quality measures for value-based care models.

For example, in diabetes, "time in range" (TIR) measured via continuous glucose monitor (CGM) is arguably a better measure of blood glucose control than the current standard for A1c. If widespread CGM use were enabled through broad reimbursement and over-the-counter

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[9] https://www.cms.gov/newsroom/press-releases/cms-building-foundational-infrastructure-digital-healthcare-ecosystem

[10] Taksler GB, Beth Mercer M, Fagerlin A, Rothberg MB. Assessing Patient Interest in Individualized Preventive Care Recommendations. MDM Policy Pract. 2019 May 27;4(1):2381468319850803. doi: 10.1177/2381468319850803.

[11] Taksler GB, Hu B, DeGrandis F, et al. Effect of Individualized Preventive Care Recommendations vs Usual Care on Patient Interest and Use of Recommendations: A Pilot Randomized Clinical Trial. *JAMA Netw Open.* 2021;4(11):e2131455. doi:10.1001/jamanetworkopen.2021.31455.

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availability (including for prediabetes), and CGM device data was systematically integrated into medical records, then TIR could be used to better personalize diabetes-related care and to replace A1c as the clinical outcome for diabetes control in value-based care structures. Incorporating TIR into quality measures, particularly measures linked to payment models, would incentivize broad adoption of TIR in clinical practice, improving both the clinical relevance of data collected and the care patients receive.

Policies that support adoption of digital health technologies (DHTs) are the key to enabling personalized, outcomes-based preventative health at scale. Virtual care management platforms, remote monitoring devices, and mobile applications that integrate multimodal health data, are the vehicles for capturing health data outside of the traditional health system. Additionally, the use of advanced DHTs can expand access to primary care in rural and under-resourced practices. Yet, many of these tools remain underutilized.[12] CMS could foster adoption by directly integrating the use of DHTs into quality measurement (e.g., HEDIS, Star ratings) and incentive payment programs (e.g., MIPs). The incorporation of DHTs into measures used for value-based payment structures would encourage providers to adopt solutions that integrate RWD and support their patients in utilizing devices and mobile apps for between visit data collection.

CMS and ASTP/ONC could also consider opportunities for piloting the use of multimodal RWD to support innovative technology product development, alternative payment mechanisms, and general testing on platforms capable of ingesting, linking, and exchanging data from EHRs, devices, patient-reported outcomes, and clinical research. For example, CMS could provide access to coverage data, pricing, and quality metrics via FHIR APIs for selected technology developers to ingest into multimodal data platforms and incorporate into patient-facing electronic applications. This would allow advanced data platforms to combine behavioral, social, and device data collected from patients with medical, pharmacy and insurance data to generate highly personalized guided workflows and more precise outcome measurement.

Notable progress has been made towards innovative uses of RWD. However, the value of these data sources can be limited by inconsistent practices involving data capture, curation, exchange, linkages, and validation. Improving processes for how RWD are captured and curated will tremendously benefit the overall utility of this ecosystem. ASTP/ONC's USCDI initiative remains a potentially untapped, yet powerful, tool for establishing normative data practices. We support the continued advancement of USCDI, and urge CMS and ASTP/ONC to support the development of robust and reliable data for a variety of stakeholder needs to improve care delivery. By enacting policies that reimburse more interoperable, more personalized, and more relevant outcome measures, value-based payment models can simultaneously incentivize better RWD *and* better care.

[12] https://www.jdpower.com/business/press-releases/2025-us-commercial-member-health-plan-study

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**Partnering on an AI Regulatory Framework for Real-World Monitoring & Payment Models**

CMS and ASTP/ONC's support of AI-enabled medical product innovation is one of the most critical opportunities to scale the digital technology ecosystem and empower patients. Verily believes that appropriate, safe and effective development and deployment of AI has enormous potential to prevent and better manage disease, and accordingly, we are building technologies that can effectively integrate multimodal health data sources. AI is already transforming how care is delivered, and as AI continues to evolve, the importance of a transparent and predictable regulatory framework for innovation, evaluation, real-world monitoring, and payment design will only grow.

An effective regulatory framework for AI, aligned with current advancements, should be developed in partnership between CMS, the FDA, and other relevant agencies and stakeholders. FDA has issued guidance documents addressing the safety and efficacy of AI-enabled medical devices, referred to as SaMD, and shared its initial thinking on the use of AI in regulatory decision making for drug and biologic products.[13] However, much uncertainty remains, specifically regarding how modifications and updates to deployed AI-enabled medical products in the post market will be monitored and reimbursed without stifling innovation or compromising patient safety. Because the scope of clinical AI deployments may span beyond conventional medical technologies and SaMD, a broader framework may be needed to allow for more predictable and effective real-world performance monitoring guidelines for deployed AI-enabled medical products.

A collaboratively developed real-world monitoring framework could be multi-purposed. It can ensure that appropriate evidence is generated for FDA post market surveillance when required, and provides a structure in which CMS can align novel payment models. Further, it could do so in ways that finally provide consumers access to their own data, enabling them to manage their own health more proactively, and thereby realizing the promise the Health Insurance Portability and Accountability Act made in 1996 (albeit when medical records were largely hard copy) to give patients meaningful access to their own health records.

In the context of regulatory approval, we suggest that the approach to real-world monitoring for deployed AI-enabled medical products be titrated to the level of risk exposed to the patient or consumer, building on how risk is classified for SaMD.[14] This could mean that a light regulatory touch, if necessary, is acceptable for lower-risk AI-enabled products, whereas higher-risk AI-enabled products might require more continuous and comprehensive data capture, mandatory reporting, or an expert in the loop to ensure ongoing safety and effectiveness. Still, the current regulatory framework may not be adequate to support the range of AI deployments

[13] www.fda.gov/medical-devices/software-medical-device-samd/artificial-intelligence-and-machine-learning-software-medical-device
[14] https://www.fda.gov/medical-devices/software-medical-device-samd/global-approach-software-medical-device

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coming to the market including those oriented towards prevention, wellness, and healthy lifestyle. Given this category of AI-enabled medical products may not fit within FDA's existing regulatory authorities, it could represent an area where a broad and refined regulatory framework is needed to ensure patient safety.

Payment models and reimbursement methodologies could align with a risk-based regulatory approach by incentivizing adoption of AI-enabled medical products that are appropriately monitored according to their classified product risk level. Harmonization of real-world performance monitoring across regulatory approval requirements and payment structures could not only help regulators keep pace with the rapid speed of innovation, but could also improve efficiencies when implementing statutorily-defined authorities across approval and payment. Additionally, this harmonization would provide more predictability and instill greater confidence so that AI technology developers continue to invest and innovate in the health sector.

In summary, FDA should consider a commensurate risk-based framework for postmarketing surveillance of safety and effectiveness for deployed AI-enabled medical products, and CMS could leverage shared insights from surveillance activities to inform and predictably implement coverage determinations or novel payment structures for AI-enabled medical products perhaps with support and input from diverse stakeholder discussions. Fundamentally, a unified, multi-stakeholder approach to innovation, performance evaluation and ongoing safety monitoring will be essential for instilling patient and provider trust in these transformative AI technologies - a critical step toward advancing the healthcare technology ecosystem.

## Closing

Thank you for the opportunity to comment on these timely and critical topics. We applaud the Administration's commitment to ensuring accessible, high-quality care for all Americans, and its focus on addressing the root causes of diseases rather than just treating symptoms. We believe the envisioned health technology ecosystem will be key to achieving these goals and transforming healthcare nationwide. We look forward to partnering with the Administration and other stakeholders to develop and implement innovative technological solutions that make this vision a reality.

**Sincerely,**

Signed by:
Vindell Washington
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**Vindell Washington, MD**
**Chief Clinical Officer**